

Skills

Providing Enteral Feedings

Checklist

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S = Satisfactory U = Unsatisfactory NP = Not Performed

Step	S	U	NP	Comments
Verified the health care provider's orders. Checked the patient's baseline weight. Reviewed lab results, such as blood glucose and electrolyte values.	☐	☐	☐	
Gathered the necessary equipment and supplies.	☐	☐	☐	
Performed hand hygiene, and provided for the patient's privacy.	☐	☐	☐	
Introduced yourself to the patient and family, if present.	☐	☐	☐	
Identified the patient using two identifiers according to agency policy. Compared these identifiers with the information on the patient's identification bracelet.	☐	☐	☐	
Asked the patient for any food allergies, and explained the procedure to the patient and ensured that he or she agreed to treatment.	☐	☐	☐	
Applied clean gloves.	☐	☐	☐	
Auscultated for bowel sounds, and assessed abdomen. Placed the patient in the high-Fowler's position, or elevated the head of the bed 30 to 45 degrees. If the patient was required to remain supine, placed him or her in the reverse Trendelenburg position.	☐	☐	☐	
Obtained the prescribed formula to administer to the patient: 1. Identified and confirmed the enteral nutrition label, and checked the expiration date. Checked the integrity of the container. 2. Administered the formula at room temperature.	☐	☐	☐	
Prepared the formula for administration: 1. Used aseptic technique when manipulating the components of the feeding system, including the formula, administration set, and connections. 2. Shook the formula container well. Cleaned the top of any canned formula with an alcohol swab before opening it.	☐	☐	☐	

3. When using an open system, poured the formula from the can into the administration bag. When filling a feeding container bag with formula, poured only the amount needed for 4 to 8 hours of feeding. Hung the bag and primed the tubing. For closed systems, connected the administration tubing to the container. Ensured the correct tubing was used. Used aseptic technique to avoid contaminating the feeding system when connecting and hanging it.

Verified tube placement, referring to the video skill “Managing a Nasogastric Tube.” Verified placement of the enteral feeding tube every 4 hours using observation of gastric aspirate, pH testing of gastric aspirate, or capnography.



1. Checked gastric residual volume (GRV) before each intermittent feeding and every 4 hours for continuous feedings.
 1. Drew 30 mL of air into a 60-mL enteral syringe, attached the syringe to the proximal end of the enteral feeding tube, and then injected the air into the tube.
 2. Drew back on the syringe slowly, and aspirated the total amount of gastric contents. If necessary, repositioned the patient to facilitate withdrawal of fluid from the tube.
 3. Noted the amount of gastric aspirate. Observed for changes in the volume of aspirate.
 4. Returned aspirated gastric contents to the stomach unless the volume exceeded 500 mL or an amount determined by the organization’s practice or practitioner’s order.
 5. Flushed the tube with 30 mL of purified or sterile water.

Used enteral devices with enteral connectors that comply with ISO standard 80369-3 (ENFit). Before attaching a feeding administration set to the feeding tube, traced the tube to its point of origin. Labeled the administration set: “Tube feeding only” or used the label provided by the manufacturer. Checked vital signs immediately after making any connection per the organization’s practice.



Labeled the bag with the tube-feeding formula type, strength, and amount. Included the date, time, and your initials. Labeled commercial enteral containers “Not for IV Use” to decrease the risk for an enteral misconnection.



Continuous or intermittent feeding with feeding bag using a pump:



1. Pinched or clamped the end of the feeding tube, removed the cap, and attached it to the tubing. Did not force connections, and avoided workarounds per the organization’s practice.
2. Checked vital signs immediately after making any connection per the organization’s practice.
3. Set the infusion rate by adjusting the roller clamp on the tubing, or attached the tubing to the feeding pump. Allowed the bag to empty gradually over 30 to 45 minutes.
4. Gradually advanced the rate of tube feedings, as ordered.
5. Capped or clamped the end of the feeding tube used for intermittent feeding when not in use.

Used purified or sterile water for flushing and medication preparation. Used sterile water for reconstituting powdered formula and for all procedures that involve immunocompromised patients.



Flushed the feeding tube. Kept the head of the bed raised until at least 1 hour after the infusion was complete.

1. Following the administration of medications or formula, cleared the tube by flushing with a minimum of 15 mL of purified or sterile water.

2. Flushed with 30 mL of water after GRV measurements.

3. Flushed with a minimum volume of 30 mL of water before and after feedings and every 4 hours during continuous feedings.

Helped the patient into a comfortable position, and made sure the head of the bed remained elevated 30 to 45 degrees.

Placed the call light within easy reach, and made sure the patient knew how to use it to summon assistance.

To ensure the patient’s safety, raised the appropriate number of side rails and lower the bed to the lowest position.

Disposed of used supplies and equipment. Left the patient’s room tidy.

Removed and disposed of gloves, if used. Performed hand hygiene.

Documented and reported the patient’s response and expected or unexpected outcomes.

Learner:

Evaluator:

Date:

Signature:

Signature: